BASELINE → TREATMENT → FOLLOW-UP

Required and conditional checks across your treatment

Required checks are shown explicitly. ECG, additional urine tests and anticoagulant tests apply only when indicated. Scans and tumour markers follow your individual cancer plan; this page does not prescribe a universal surveillance interval. Confirm who books and reviews each test.

Check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver	Required baseline blood count, kidney and liver tests	Required before each cycle; results reviewed before treatment	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed
DPD / fluoropyrimidine safety	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
Nerves / cold sensitivity	Record any pre-existing numbness or difficulty with daily tasks	Neuropathy assessment every cycle for this regimen	Report persistent tingling, trouble with buttons or walking; avoid cold triggers	Oxaliplatin may be changed separately after clinical review	Nerve symptoms can persist; arrange follow-up if unresolved
Heart assessment	Review cardiac history; baseline ECG if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series
Response / scans / tumour markers	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00555 v7 · official regimen

THE WHOLE TREATMENT JOURNEY

FOLFOXIRI: treatment and checks

Metastatic colorectal cancer

Day 1: irinotecan, oxaliplatin and folinic acid, followed by a 48-hour fluorouracil pump. Repeat every 14 days while treatment is helping and side effects remain acceptable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Review / change	After treatment
Blood counts / kidneys / liver NCCP 00555	Required baseline blood count, kidney and liver tests	Required before each cycle; results reviewed before treatment	Earlier tests if unwell, dehydrated or toxicity develops	Recovery and results guide delay or dose adjustment	Follow unresolved abnormalities as agreed
DPD / fluoropyrimidine safety NCCP 00555	DPD test before fluorouracil or capecitabine unless already tested	The result guides the starting plan; it does not exclude all severe toxicity	Unexpected severe early mouth sores, diarrhoea or illness needs urgent assessment	The team reviews suspected severe reactions before further treatment	Keep the result and any toxicity history in your records
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Response / scans / tumour markers NCCP 00555	Agree a baseline disease assessment and treatment aim	Review symptoms, function and clinical benefit	New symptoms may require earlier review	Scans and tumour markers follow the consultant-directed cancer plan	Agree next treatment or follow-up; there is no universal scan interval in this regimen

What each check is for

1

14-day treatment cycle

Day ward → home pump → review

FOLFOXIRI combines three chemotherapy medicines with folinic acid to treat colorectal cancer that has spread. The aim is cancer control, symptom relief and, for selected people, a response that allows further local treatment after specialist review.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

Sources: NCCP 00555

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.33